



Philippine Integrated Disease
Surveillance and Response

Case Investigation Form
Measles/Rubella
(ICD 10 Code: B05; B06)

Version 2019



Name of DRU:		Type: ☐ RHU ☐ CHO ☐ Gov't Hospital ☐ Private Hospital ☐ Clinic	
DRU Complete Address: <u>House No.</u> <u>Street/Purok</u>		☐ Gov't Laboratory ☐ Private Laboratory ☐ Airport/Seaport	
<u>Barangay</u>	<u>Muni/City</u>	<u>Province</u>	<u>Region</u>

I. PATIENT INFORMATION

Patient Number	EPI ID	Patient's First Name	Middle Name	Last Name
Sex: ☐ Male ☐ Female		Date of Birth: <u>MM</u> / <u>DD</u> / <u>YY</u>	Age: ☐ Days ☐ Months ☐ Years	Current Address: (Specify House/Lot #, Street/Purok/Subdivision, Barangay, Municipality/City, Province, Region)
Pregnant? ☐ Y ☐ N ☐ U If Yes, weeks of pregnancy <u> </u>				
Patient admitted? ☐ Y ☐ N	Date Admitted/Seen/Consult	<u>MM</u> / <u>DD</u> / <u>YY</u>	Permanent Address: (Specify House/Lot #, Street/Purok/Subdivision, Barangay, Municipality/City, Province, Region)	
Is the case a member of Indigenous Group? ☐ Y ☐ N If YES, specify: <u> </u>				
Name of parent/caregiver:		Contact Nos.:		
Date of Report: <u>MM</u> / <u>DD</u> / <u>YY</u>	Name of reporter:	Contact Nos.:		
Date of Investigation: <u>MM</u> / <u>DD</u> / <u>YY</u>	Name of investigator/s:	Contact Nos.:		

II. CLINICAL DATA

Fever: ☐ Y ☐ N Date onset: <u> </u> / <u> </u> / <u> </u>	Arthralgia/arthritis: ☐ Y ☐ N Swollen lymphatic nodules: ☐ Y ☐ N If Y, specify location: ☐ cervical ☐ sub-occipital ☐ post-auricular ☐ others, specify <u> </u>	Are there any complications? ☐ Y ☐ N If YES, specify: <u> </u> Other symptoms: <u> </u> Working/Final Diagnosis: <u> </u>
Rash: ☐ Y ☐ N Date onset: <u> </u> / <u> </u> / <u> </u>		
Cough: ☐ Y ☐ N Koplik sign: ☐ Y ☐ N Runny nose/coryza: ☐ Y ☐ N Red eyes/conjunctivitis: ☐ Y ☐ N		

III. VACCINATION HISTORY AND VITAMIN A SUPPLEMENTATION

Patient received measles-containing vaccine (MCV)?	☐ Y	☐ N
If Yes, indicate the number of doses whichever is applicable:	MV <u> </u>	MR <u> </u> MMR <u> </u>
Date last dose received MCV: <u> </u> / <u> </u> / <u> </u>		
Measles vaccine received validated through: ☐ Vaccination Card ☐ Logsheet ☐ By recall ☐ <u> </u> (others, specify)		
Was vaccination received during special campaigns?	☐ Y	☐ N
If patient did not receive any MCV, state the reason/s:		
☐ Mother was busy	☐ Child was sick	☐ Forgot schedule
☐ Against belief	☐ No vaccine available	☐ Other reasons, specify <u> </u>
☐ Medical contraindication	☐ Vaccinator not available	
☐ Fear of side effects	☐ Not eligible for vaccination	
Was the patient given Vitamin A during this illness?	☐ Y	☐ N

IV. EXPOSURE HISTORY

With history of travel within 23 days prior to onset of rash? : ☐ N ☐ Y If YES, specify place and timing: Place of travel: <u> </u> Date of travel: <u> </u> / <u> </u> / <u> </u> ☐ <7 days from rash onset ☐ 7-23 days from rash onset
*Was there contact with a confirmed Measles case 7-23 days prior to rash onset? ☐ N ☐ U ☐ Y
*Was there contact with a confirmed Rubella case 7-23 days prior to rash onset? ☐ N ☐ U ☐ Y
If YES, name of contact: <u> </u> Place of residence: <u> </u> Date of contact <u> </u> / <u> </u> / <u> </u>
Tick the type of place where exposure probably occur: ☐ Day care ☐ Barangay ☐ Home ☐ School ☐ Health Care Facility ☐ Dormitory ☐ Others, specify <u> </u>
*Are there other known cases with fever and rash (regardless of presence of 3 C's) in the community? ☐ Y ☐ N ☐ U

• Note: If the answer to any of the two questions was YES, coordinate with the ESU for validation and field investigation can't at the back

Deliberately providing false or misleading, personal information on the part of the patient, or the next of kin in case of patient's incapacity, may constitute non-cooperation punishable under the Republic Act No. 11332.

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V. LABORATORY TESTS

Specimen collected (Put ✓ in the box Provided)	If YES, Date Collected	Date sent to RITM	Date received in RITM (to be filled up by RITM)	Measles IgM Result	Rubella IgM Result	Virus Isolation Result	PCR Result
☐ Serum	___/___/___	___/___/___					
☐ Dried Blood Spot	___/___/___	___/___/___					
☐ Oropharyngeal/ Nasopharyngeal swab?	___/___/___	___/___/___					

VI. FINAL CLASSIFICATION

- | | |
|---|--|
| ☐ Laboratory Confirmed Measles | ☐ Laboratory Confirmed Rubella |
| ☐ Epi-linked Confirmed Measles | ☐ Epi-linked Confirmed Rubella |
| ☐ Measles Compatible | ☐ Discarded Non Measles/Rubella |

VII. SOURCE OF INFECTION

- ☐ Endemic
☐ Imported
☐ Import-related
☐ Unknown

VIII. OUTCOME: ☐ Alive ☐ Died

Date died: ___/___/___

FINAL DIAGNOSIS: _____

CASE DEFINITION AND CLASSIFICATION

CASE DEFINITION

Suspected case: Any individual, regardless of age, with the following signs and symptoms:

- fever (38°C or more) or hot to touch; and
- Maculo-papular rash (non-vesicular) AND
- at least one of the following: cough, coryza (runny nose), or conjunctivitis (red eyes)

CASE CLASSIFICATION

Laboratory-confirmed Measles: a suspected measles case that has been confirmed by a proficient laboratory as positive for Measles IgM antibodies and/or positive for measles virus isolation or Polymerase Chain Reaction (PCR)

Epidemiologically Linked Confirmed Measles: a suspected measles case that has not been confirmed by a laboratory but was geographically and temporally related with dates of rash onset occurring between 7 and 23 days apart from a laboratory-confirmed case or another epidemiologically confirmed measles case

Clinically Compatible Measles: a suspected measles case, for which no adequate clinical specimen was taken and the case has not been linked epidemiologically to a laboratory-confirmed case of measles or other communicable disease OR laboratory confirmation is still pending

Laboratory-confirmed Rubella: a suspected measles case with a positive laboratory test results for rubella-specific IgM antibodies or other laboratory test method

Epidemiologically Linked Confirmed Rubella: a suspected case who has direct contact with another laboratory confirmed rubella case with rash onset occurred 12-23 days before the present case

Non-Measles/Rubella Discarded Case: a suspected case that has been investigated and discarded as a non-measles (and non-rubella) when any of the following are true:

- negative laboratory testing in a proficient laboratory on an adequate specimen collected during the proper time period after rash onset
- epidemiological linkage to a laboratory confirmed outbreak of another communicable disease that is not measles/rubella
- confirmation of another etiology

SOURCE OF INFECTION:

Endemic: a confirmed measles case acquired the infection within the country wherein the chain of measles virus transmission is continuous for ≥12 months

Imported: a returning traveler or visitor exposed to measles outside the country during the 7–23 days prior to rash onset and supported by epidemiological or virological evidence.

Import-related: a locally acquired infection that occurs as part of a chain of transmission originating from an imported case as supported by epidemiological or virological evidence.

Unknown: a confirmed case for which no epidemiological or virological link to importation or endemic transmission can be established after a thorough investigation.

LABORATORY CONFIRMATION:

- Positive serologic test result for anti-measles IgM antibodies
- Fourfold rise in anti-measles IgG antibodies in acute and convalescent serum
- Isolation of measles virus
- Dot immunobinding assay
- Polymerase chain reaction (PCR) testing for measles nucleic acid

Therapeutic Dosage of Vitamin A for Measles cases:

- 50,000 IU for children <6 months old
- 100,000 IU for children 6 to 11 months old
- 200,000 IU for children 12 to 71 months old

Note: The therapeutic dosage of Vitamin A for measles cases should be given upon diagnosis regardless of when the last dose of vitamin A capsule was given.

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